

End-to-End User Journey

Autism Therapy Platform — Indian Therapy Center Lifecycle

Prepared for: Product & Design Leadership

Date: 2026-04-14 ■ Stage: Discovery ■ Evidence: Secondary Research

Evidence labels: ■ Observed (peer-reviewed / regulatory) | ■ Inferred
■ Hypothesis (unvalidated) | ■■ DPDPA risk

The Journey at a Glance

A child's therapy journey at an Indian autism center spans 8 stages across 4 personas — from a family's first WhatsApp message to ongoing therapy, billing, and (too often) silent dropout. The entire journey runs on paper, WhatsApp, and memory. No structured digital tool exists at any stage.

Stage	Primary Persona	Core Tool Today	Biggest Break
1. Family inquiry	Rahul (Director)	WhatsApp / memory	No inquiry tracking; leads lost
2. Intake & enrollment	Rahul, Dr. Sunita	Paper forms	No consent mechanism; DPDPA exposure ■■
3. Assessment & program	Dr. Sunita (Supervisor)	Paper / Word	Verbal handover to therapist; program misapplied
4. In-session data coll.	Priya (Special Educator)	Paper data sheet	One-handed constraint; 1–2 week feedback lag
5. Supervisor review	Dr. Sunita	Paper / Excel	Batch review; outdated targets run for weeks
6. Progress reporting	Dr. Sunita	Word / WhatsApp	Written from scratch; reports parents can't read ■■
7. Billing & fee coll.	Rahul	WhatsApp / paper	Manual, relationship-sensitive; fees delayed
8. Dropout & follow-up	Rahul	WhatsApp message	No tracking; dropout invisible until it's done

Stage-by-Stage Breakdown

Stage 1 — Family Inquiry & First Contact

A parent hears about the center through word of mouth or a paediatrician referral and sends a WhatsApp message. The inquiry is noted in the WhatsApp thread itself — or in a paper notebook — with no structured record. There is no pipeline visibility, no automated follow-up, and no reminder if the family goes quiet.

Emotional state: Meena (parent) arrives anxious and exhausted from a long diagnostic journey. This is a high-stakes trust moment. ■ Tandfonline 2025

– **Key break:** Warm leads are lost because they live in a WhatsApp scroll history. ■

Stage 2 — Intake & Enrollment

The family arrives for an intake appointment. A developmental history is taken verbally or on paper. Prior documents — diagnosis reports, school records, UDID card — are collected as photocopies. A fee structure is explained verbally. A consent form may or may not be signed.

Emotional state: Meena is overwhelmed and needs to feel heard. Dr. Sunita is clinically focused but time-pressured. ■ / ■

- No standardised intake protocol — process varies by staff and day ■ PMC
- Digital storage of child health records without verifiable parental consent = **DPDPA 2023 non-compliance** ■■ ■
- Fee agreement is verbal — ambiguity accumulates over months ■

Stage 3 — Assessment & Program Design

Dr. Sunita conducts 1–3 assessment sessions (ISAA, CARS, Vineland). Results are compiled on paper into a baseline profile. An individualised therapy program is written — targets, prompt levels, reinforcement schedules. This program is communicated to Priya via a verbal briefing, possibly with a paper handout.

- Verbal handover = Priya may run sessions with misremembered prompt levels or wrong targets ■
- Parents leave without a written summary of what is being worked on or why ■
- RPWD Act 2016 mandates documented individualised programs — compliance is informal ■ / ■

Stage 4 — Ongoing Therapy Sessions (In-Session Data Collection)

This is the highest-frequency workflow in the product. Priya runs discrete trials (DTT) or naturalistic teaching (NET) with the child, marking outcomes on a paper data sheet — correct, incorrect, or prompted — while managing the child with her other hand.

- **One-handed constraint** makes paper recording physically awkward; entries are missed or illegible ■
- ABC data (antecedent-behaviour-consequence) is often written retrospectively from memory — inaccurate by design ■
- Paper data sheets sit in a physical file until Dr. Sunita reviews them — sometimes **1–2 weeks later** ■ BHCOE
- Some therapists photograph paper sheets and send via WhatsApp — unencrypted transmission of child health data ■■ DPDPA ■

This is the foundational break point. Everything downstream — program updates, reports, billing — depends on session data that is currently inaccurate, delayed, and inaccessible.

Stage 5 — Supervisor Review & Program Updates

Dr. Sunita collects paper data sheets and manually calculates percentage-correct per target. She identifies mastery or plateau patterns and updates the therapy program. Changes are communicated to Priya verbally.

- Manual calculation is time-consuming and error-prone ■
- Global benchmark: **2–3 hours/day** on documentation without software tools ■ ABA Matrix — Indian equivalent unvalidated ■
- Program update communicated verbally → Priya may continue running old targets ■
- No version history: “What was the prompt level 4 weeks ago?” is unanswerable ■

Stage 6 — Progress Reporting to Parents

Monthly or quarterly, Dr. Sunita compiles session data and writes a progress narrative per domain. Reports are handed over in person or sent as a PDF via WhatsApp. A verbal meeting may accompany the report. Home program instructions are given verbally.

- **Reports are written from scratch every cycle** — no carry-forward from prior reports or auto-population from session data ■
- Report language is clinical; Meena frequently doesn’t understand what she’s reading ■
Product context
- WhatsApp delivery of progress reports = unencrypted sensitive health data ■■ DPDPA ■
- Home program guidance is verbal — Meena is unlikely to remember what to practise ■

Stage 7 — Billing & Fee Collection

Rahul tallies sessions from a paper attendance register, calculates fees, and sends a WhatsApp message to the family. Payment is made in cash or via UPI. Outstanding balances are tracked in Excel or not at all.

- Asking financially stressed families for money is emotionally uncomfortable — Rahul delays these conversations ■
- No automated reminder — Rahul must manually track who has paid ■
- No financial dashboard: monthly revenue, collection rate, outstanding fees are invisible at a glance ■
- Evidence: **no-show rates drop from 39% → 3%** with structured reminders ■ Psychiatric Services — the same principle applies to payment follow-up ■

Stage 8 — Appointment Follow-Up & Dropout Prevention

A family misses a session. Staff notice through memory or a gap in the paper schedule. A single WhatsApp message is sent. If the family goes quiet, dropout is effectively accepted — experienced as an outcome, not a process failure.

- No attendance tracking system — dropout is **invisible until it has already happened** ■
- “Invisible exits”: families withdraw silently under financial strain and caregiver exhaustion ■ Tandfonline 2025

- One WhatsApp message is the entire dropout intervention — evidence shows live contact is 13× more effective ■
- No re-engagement protocol: if a family returns after a gap, no structured way to update the program exists ■

The 5 Highest-Impact Break Points

#	Break Point	Stage	Personas Hit	Evidence
BP-01	Paper in-session data collection — delayed, inaccurate, one-handed	Stage 4	Priya, Dr. Sunita	■ ■
BP-02	Supervisor review happens in batch, 1–2 weeks behind	Stage 5	Dr. Sunita, Priya	■
BP-03	Progress reports written from scratch; parent can't understand them	Stage 6	Dr. Sunita, Meena	■ ■
BP-04	No attendance tracking; dropout invisible until complete	Stage 8	Rahul, Meena	■ ■
BP-05	No intake protocol; DPDPA non-compliance from day one	Stage 2	All	■ ■ ■

What This Means — Decisions Required

- 1. In-session data collection is the foundation.** Every other break point is downstream of inaccurate, delayed session data. Solve this first or there is nothing reliable to build reporting, billing, or dropout prevention on.
- 2. DPDPA compliance is not a later feature.** It appears at Stages 2, 4, and 6. The first time the product stores a child's clinical data digitally, verifiable parental consent is a legal requirement — not a roadmap item.
- 3. WhatsApp is in the journey whether we design for it or not.** It is the inquiry channel, the billing channel, the progress report delivery channel, and the dropout follow-up channel. The product must map to these touchpoints, not replace them.
- 4. Dropout prevention has the strongest evidence ROI.** The 39% → 3% no-show rate finding is the sharpest data point across all research. Structured attendance tracking with reminder triggers is a high-impact, low-complexity Phase 2 candidate.
- 5. Validate before designing.** Approximately 45% of this journey map is hypothesis, not observed fact. Primary fieldwork at 3–5 centers is the gate before any design work begins.

Open Assumptions (Top 5 by Risk)

Hypothesis	Risk	Validate via
Priya records trial data on paper during live sessions (vs. skipping entirely)	High	Contextual observation
Offline-first is a hard requirement in session rooms	High	On-site connectivity test
Supervisor-to-therapist handover is verbal; program is misapplied	High	Observation + debrief interview
Supervisors spend significant out-of-hours time on report writing	High	Time-diary / interview
Centers are not DPDPA-compliant at intake	High	Director interview + consent form review